VegaHealth

Baptist Health

Statement of Work
AI Infrastructure & Monitoring

October 9, 2026

SOW No: 2026-BHKI-01

This document is the property of Vega Health Inc. and must not be disclosed, duplicated, or used—in whole or in part — for any purpose other than to evaluate this Statement of Work. This document contains trade secret and proprietary commercial or financial information. Accordingly, no portion of this document should be released without consulting Vega Health Inc. This information is contingent on Vega Health and Baptist Health reaching mutually agreeable terms and conditions and upon acceptance of any limitations described herein.

VegaHealth

October 9, 2026

To: Baptist Health

Subject: AI Infrastructure and Monitoring Statement of Work (SOW)

To the Baptist Health Team:

Vega Health is excited to have the opportunity to partner with Baptist Health to install the Vega Health Platform and implement monitoring capabilities for three (3) Use Cases over an initial six-month period. Baptist Health has identified the following as potential in-scope Use Cases: Gleamer BoneView, an Epic Generative AI product, and a LeanTaaS model.

Rooted in over a decade of innovation and collaboration at Duke Health, Vega Health was founded by healthcare leaders and technologists with experience developing, integrating, monitoring, and scaling validated health AI technologies in real-world healthcare settings.

As part of this pilot, Vega Health will present Baptist Health (collectively, the “Parties”) with separate addenda to be entered into by the Parties pursuant to this SOW (each, a “Use Case Addendum”), which outline the specific work and efforts anticipated to onboard and implement monitoring capabilities for each Use Case. Each Use Case Addendum will set forth the specific technical, data, and resource requirements reasonably necessary to support the monitoring capabilities.

This SOW is structured as a six-month engagement that is part of the Strategic Collaboration outlined in the Master Services Agreement (MSA). Our intent with this proposal is to begin working together soon, so that we can demonstrate the value that the Platform and monitoring capabilities may provide to your organization. As part of the Strategic Collaboration structure, the Parties will establish an Executive Steering Committee (“ESC”) that will guide the overall relationship, approve the completion of Deliverables, and serve as the decision-makers for additional Use Cases. New Use Cases can also be brought forward to the ESC at any time.

We are grateful for the opportunity to collaborate with Baptist Health and look forward to supporting your team throughout this work. If you have any questions about this SOW, please do not hesitate to contact our team at Mark.Sendak@VegaHealth.com. Thank you and we look forward to your response.

Sincerely,

Mark Sendak, CEO & Co-Founder
Mark.Sendak@VegaHealth.com

I. Overview of the Statement of Work

This Statement of Work ("SOW") between [Baptist Healthcare System] ("Baptist Health") and Vega Health Inc. ("Vega Health"), a Delaware corporation, is entered into under, and incorporates by reference, the Master Services Agreement between Vega Health and Baptist Health, dated as of *[___]* ("MSA"). Under this SOW, Vega Health will install the Vega Health Platform within Baptist Health's environment, onboard three (3) existing AI models deployed or licensed by Baptist Health (each, a "Monitored Model"; references in this SOW to an "AI product," "AI technology," or "model" refer to a Monitored Model), and implement monitoring capabilities for each Use Case. A Monitored Model that is licensed to Baptist Health by a third-party vendor is not a Third-Party Model (as defined in the MSA), and Baptist Health remains responsible for maintaining all rights and licenses necessary for Vega Health to access the inputs, outputs, and related data of each Monitored Model as contemplated by this SOW, consistent with MSA §8.2. The installation of the Vega Health Platform (referred to as "Platform" throughout) is included in this first SOW to support Baptist Health's use of the monitoring capabilities on an ongoing basis The total charge for this SOW is $500,000, which includes the Platform installation, model onboarding, and the implementation of Use Case-specific monitoring capabilities.

This SOW is effective upon execution by both Parties. As provided in MSA §4.1(a), in the event of any conflict between this SOW and the MSA, this SOW shall govern with respect to the subject matter set forth herein. Capitalized terms not defined herein have the meanings given to them in the MSA. The services delivered by Vega Health are limited to those explicitly described in this SOW and any separate executed Use Case Addendum. Any other work requires separate written authorization by both Parties.

The pilot will begin on the day of the first ESC meeting ("Pilot Start Date") and continue for six (6) months ("Pilot Term"), unless earlier terminated under MSA §12.2 or extended by mutual written agreement by the Parties.

After this SOW has been executed and before the Pilot Start Date, the Parties will make a good faith effort to complete a specific set of activities ("Pre-Pilot Preparation") to support the timely delivery of Deliverables (as defined in Section IV) within the original Pilot Term. The specific set of activities will be shared upon execution of this SOW. The activities may include but not be limited to:

- Vega Health will conduct initial technical discovery, including but not limited to, providing documentation and facilitating working sessions on the technical integrations and resource requirements for Platform installation and outlining potential risks and/or anticipated next steps for each Use Case (e.g., additional contracting requirements to enable data integrations with other third-party AI products).
- Baptist Health will provision the appropriate resources, including but not limited to, accounts and technical environments (e.g., staging, production). The initial technical resource dependencies and requirements have been outlined in Section V.

Any delays or incompletion of Pre-Pilot Preparation requirements within the timeline outlined will be addressed as described under Section IVA. The Pilot Start Date will be set to occur once Pre-Pilot Preparation activities have been completed, or no later than thirty (30) days after the execution of this SOW, whichever comes first. If the Pilot Start Date occurs before Baptist Health has completed its Pre-Pilot Preparation responsibilities, the Projected Timing of affected Milestones and the Pilot Term will be equitably extended to the extent of any resulting delay.

II. Milestones

During the Pilot Term, Vega Health will support Baptist Health to achieve the following Milestones as part of this SOW:

- Install the Platform to facilitate secure data exchange between local data sources and the Platform.
- Implement monitoring capabilities for three (3) Use Cases, as mutually agreed upon by the Parties in separate, written Use Case Addenda. AI products that may be selected for this SOW include Gleamer BoneView, an Epic Generative AI product, and LeanTaaS model. The Parties will decide on and outline the specific measures that will be tracked and configured for continuous monitoring for each Use Case-specific Monitoring Plan. The potential types of data that may determine the scope and feasibility of monitoring for each proposed Use Case have been outlined in Section V.
- Support Baptist Health teams' operational readiness to use the Platform and in-scope Use Case monitoring capabilities through the documentation and trainings described in Section IV.

The Platform will be installed within Baptist Health's environment. Data ownership, permitted use of PHI, and the return or destruction of data are governed by §5.8, §6.3–6.7, and §12.3 of the MSA and the Business Associate Agreement (BAA). §7 of the MSA ("AI Clinical Governance") will apply to this SOW, which includes Terms for Validation and Performance Monitoring, Human Oversight, and Regulatory Monitoring. Consistent with MSA §§7.2 and 9.2, monitoring capabilities are intended to support, and not replace, Baptist Health's professional judgment, and Baptist Health retains responsibility for clinical and operational decisions, including responses to any warnings or alerts. Vega Health does not warrant the accuracy, performance, or safety of any Monitored Model.

III. Use Case Selection

Baptist Health stakeholders have identified Gleamer BoneView as the first proposed Use Case. The additional two (2) Use Cases will be considered in partnership with the ESC and will be determined upon mutual written agreement by both Parties, at the latest, within sixty (60) days of the Pilot Start Date. The proposed in-scope Use Cases described in this SOW may be subject to change based on Baptist Health's priorities and technical discovery sessions. Other Use Cases may be considered and decided on by Baptist Health, and then agreed upon for execution by mutual agreement between the Parties.

Before work begins for any Use Case, the Parties will execute a separate Use Case Addendum as part of this SOW and as described in MSA §4.1(a). For each Use Case, the Use Case Addendum will document the model that is in-scope, as well as the projected dependencies, data integrations, data requirements, and staffing. Once the Use Case Addendum has been executed, the Parties will develop a Use Case-specific Monitoring Plan to align on the proposed monitoring specifications (e.g., metrics, thresholds, dashboards, alerts, alert response, escalation pathway, operational ownership).

The Parties will begin defining and confirming the Use Cases on the Pilot Start Date and aim to complete the selection as quickly as reasonably possible, so that engineering resources can be allocated efficiently across Use Cases and any overlap identified during initial scoping can be

used to streamline the build. The Platform installation may proceed in parallel with Use Case onboarding and the implementation of monitoring capabilities.

If the Parties cannot reach a mutually agreed upon scope for any Use Case within sixty (60) days of the Pilot Start Date, Vega Health will not be responsible for completing Use Case onboarding and the configuration of monitoring capabilities within the original Pilot Term. Delays that may impact the original Pilot Term will be addressed according to the process outlined in Section IVA of this SOW.

IV. Deliverables

The following table outlines the deliverables within the scope of this SOW ("Deliverables"). The Deliverables will be considered accepted and completed based on the Acceptance Criteria outlined in Section IVB. If the Parties identify additional critical dependencies as the work progresses for each Use Case, including integration requirements with third-party vendors, the Project Plan and applicable Use Case-specific Monitoring Plan will be amended by mutual written agreement between the Parties. If a critical dependency cannot be resolved within a reasonable timeline, either Party may escalate it to the ESC and recommend re-scoping the impacted Use Case.

Based on an initial evaluation of each Use Case and execution of each separate Use Case Addendum, the Project Plan will be updated to include additional details (e.g., timeline, resource requirements) and confirmed with the ESC. Additionally, as part of the Project Plan, a RACI matrix will be developed in coordination with the Baptist Health team and presented to the ESC for formal approval.

Figure 2. Deliverables and Details

Deliverable	Details
Governance Structure	Support the establishment of the ESC per MSA §3 and facilitate, at minimum, monthly strategic sessions on the partnership and pilot-specific goals, Use Case selection, and long-term AI strategy
Project Plan	Deliver detailed workplan, staffing, roles and responsibility assignment matrix (RACI), and anticipated risks for this SOW
Technical Documentation Report	Deliver Technical Documentation Report based on working sessions to outline the data sources, data mapping, information flows, and infrastructure required to install the Platform (this will require the facilitation of technical discovery sessions)
Use Case Addendum	Execute a separate Use Case Addendum per confirmed and selected Use Case with the appropriate specifications and details, including but not limited to, any Use Case-specific deliverables, workstreams, and dependencies
Vega Health Platform Installation ("Platform Installed" as defined in Section IVB)	Install the Platform software on resources managed by Baptist Health Integrate the Platform to align to custom configurations of Baptist Health computing resources, as well as install and configure site adapters designed to support secure data exchange consistent with applicable HIPAA requirements, and exchange test data per the industry-standard data integration specifications (e.g. HL7v2, FHIR)

Deliverable	Details
Use Case Monitoring Plan	Deliver a Use Case-specific Monitoring Plan, including details such as metrics, thresholds, dashboards, and alerting requirements
Use Case 1: Onboarding and Monitoring ("Monitoring Implemented" as defined in Section IVB)	Onboard the in-scope model onto the Platform, including but not limited to, integration of the data pipelines and data feeds Implement monitoring capabilities according to appropriate Monitoring Plan
Use Case 2: Onboarding and Monitoring ("Monitoring Implemented" as defined in Section IVB)	Onboard the in-scope model onto the Platform, including but not limited to, integration of the data pipelines and data feeds Implement monitoring capabilities according to appropriate Monitoring Plan
Use Case 3: Onboarding and Monitoring ("Monitoring Implemented" as defined in Section IVB)	Onboard the in-scope model onto the Platform, including but not limited to, integration of the data pipelines and data feeds Implement monitoring capabilities according to appropriate Monitoring Plan
Operational Readiness Documentation and Trainings	Provide supporting documentation and conduct operational readiness trainings for the Platform and each Use Case

A. Managing Delays

As part of regular status updates, any delays that require additional resources or a change in priorities will be raised with the Executive Champion and the Vega Health Engagement Manager, who will work together to resolve them. Matters they cannot resolve will be escalated to the ESC, which will make the final decision; provided that any change to scope, Fees, or Vega Health's resource commitments will require a written amendment under Section VII. Both Parties will use commercially reasonable efforts to adjust resources and priorities as needed to keep to the initial project timeline and avoid affecting the timing of Deliverables.

B. Acceptance Criteria

Once a Deliverable is completed, Vega Health will present the Deliverable to the ESC for approval at the next scheduled ESC meeting or, at the discretion of the Executive Champion and Vega Health, at an ad-hoc meeting, together with an acceptance package including, as applicable, test evidence, security validation results, documentation, and/or a completed operational readiness checklist. If the ESC identifies specific non-conformities with the applicable acceptance criteria set forth in this Section IVB, it will notify Vega Health in writing within ten (10) business days after presentation, and Vega Health will address them and re-present the Deliverable. Approval will be recorded in writing and will not be unreasonably withheld, conditioned, or delayed. A Deliverable will be deemed accepted if the ESC does not provide written notice of specific non-conformities within such ten (10) business day period, or if Baptist Health uses the Deliverable in production.

Acceptance criteria for the Vega Health Platform installation, as well as onboarding and monitoring for each Use Case are defined below.

Platform Installed means: (a) the Platform is installed and deployed within the target environments specified in the Technical Documentation Report; (b) integration with and verification of site adapter connectivity to in-scope data sources; (c) test data exchange is completed and documented; (d) supporting documentation is delivered; and (e) delivery of technical runbook for technical support by the Baptist Health systems team.

Monitoring Implemented means, for a Use Case: (a) a Monitoring Plan for the Use Case has been completed defining (i) minimum set of metrics to be monitored, (ii) thresholds and rules for generating warnings/alerts when metrics are out of bounds, (iii) Baptist Health individual responsible for first-line response to warnings/alerts for each metric, and (iv) escalation pathways; (b) Monitoring Plan is approved by the ESC or their designated representative; (c) production data pipelines have been built to ingest the data needed for monitoring of each Use Case, to the extent such data is made available by Baptist Health and its vendors; (d) all required metrics in the approved Monitoring Plan are calculated and displayed in a dashboard within the Vega Health Platform; and (e) thresholds are configured and alerts are configured to be delivered to intended recipients according to the approved Monitoring Plan.

Acceptance criteria for all other Deliverables have been included below:

Deliverable	Acceptance Criteria
Governance Structure	• Identify ESC membership • Facilitate, at minimum, monthly ESC meetings
Project Plan	• Present the Deliverable to the ESC • ESC approves the document via writing
Technical Documentation Report	• Present the Deliverable to the ESC • ESC approves the document via writing
Use Case Addendum	• Present the Deliverable to the ESC and Use Case Sponsor • ESC approves the document via writing • Authorized signatories execute the Addendum
Monitoring Plan	• Present the Deliverable to the ESC and Use Case Sponsor • ESC approves the document via writing
Operational Readiness Documentation and Trainings	• Present supporting documentation on the Platform and each Use Case to the ESC and to each Use Case Sponsor • Conduct, at minimum, one operational readiness training for the Platform and each Use Case with the relevant stakeholders (e.g., end-users, Use Case Sponsor) • ESC approves the documents and trainings via writing

V. Technical Dependencies & Requirements

The following preliminary technical dependencies have been identified; others may emerge during the Pilot Term and will be included in the Technical Documentation Report and Project Plan, as appropriate:

- Compute resources within Baptist Health's cloud or on-premise environment, including relational databases, virtual machines and/or bare metal support for servers that support

a Kubernetes environment, adequate number of processors and random-access memory

- Member of the Baptist Health team to install and maintain the necessary infrastructure as code (IaaC) to provision, deploy, and upgrade the Platform software as necessary
- Large storage volumes (Blob storage) for model output and associated resources
- Configuration and support for access to HIPPA-compliant local or external Large Language Model (LLM) (if applicable)
- Baptist Health support to configure networking (local ingress and external egress), DNS, and VPN access (if applicable)
- Collaboration with an authorized representative, or a guest account, for installation, troubleshooting, and assistance needed for support
- Coordination and facilitation by Baptist Health and appropriate cooperation of vendors, as applicable, for monitored AI technologies that require vendor support as identified by Vega Health (e.g., Gleamer, LeanTaaS)

Consistent with MSA §9.3, Vega Health will use LLMs that Baptist Health makes available under its own enterprise agreement and BAA (for example, models available through Microsoft Foundry under Baptist Health's Microsoft agreement), and Vega Health is not responsible for the availability, performance, or outputs of such LLMs or the acts or omissions of any third-party vendor of a Monitored Model. Securing LLM access is part of the Pre-Pilot Preparation in Section I.

Vega Health will require direct access and service accounts to Baptist Health data sources, as well as dedicated time with Baptist Health personnel:

- For each Use Case, Vega Health will provide a list of necessary FHIR resource identifiers (URN/URI). A Baptist Health Epic integration resource will need to configure these endpoints, grant the listed entitlements, create an internal Epic user account (with credentials for Vega Health), and configure the test and production Epic URLs
- Baptist Health will need to identify the HL7 message broker (vendor) providing access to HL7 feeds (e.g., CorePoint) and the internal employee managing third-party access, who will configure staging and production resources to provide HL7 messages

Vega Health will require the ability to automatically retrieve relevant inputs, outputs, and other related artifacts necessary for implementing monitoring capabilities:

- Access may be via API, automated retrieval from a database or Azure blob storage, or any other method that can be reliably and repeatably performed without manual human intervention to transfer data
- Exact requirements will depend on the approved Monitoring Plan for each Use Case but may include raw EHR inputs, user inputs or interactions with the model that impact model outputs, raw structured or text outputs from models, revised outputs after user intervention (e.g., edited notes generated by physicians based on model outputs), telemetry (e.g., latency) related to model outputs, and any other data deemed necessary
- Availability of required data may require modifications to the appropriate Monitoring Plan to mitigate the gaps or may restrict which models can be chosen. For example, an Epic Generative AI product that cannot be configured to log model outputs prior to a physician revising the outputs for incorporation into a note cannot be monitored for raw output quality. Based on the proposed Use Cases that may be in-scope for this SOW:

- BoneView: Monitoring will be restricted to processing structured and text model outputs and related data. Image processing and image-based monitoring are out of scope for the Platform. The assumption is that at least the following data will be available programmatically for use in monitoring: structured/text output from the BoneView model (e.g., semi-structured list of potential diagnoses identified by the model in an image), draft and final diagnoses/radiology report from the physician, and telemetry from the BoneView tool (e.g., number of images processed, model latency, etc.). Any of this data that is not available programmatically will reduce the feasible scope of the Monitoring Plan.
- Epic Generative AI product and LeanTaas model: Monitoring scope will be dependent on data availability for the selected model and Use Case. The feasibility of the monitoring scope will be assessed as part of the Use Case selection process and may influence final model selection. The appropriate Monitoring Plan may be restricted by what is feasible based on data availability.

VI. Roles and Responsibilities

The engagement will be delivered through a partnership between Baptist Health and Vega Health resources. The resource estimates below will be refined in the Project Plan and each Use Case Addendum, as applicable.

A. Executive Steering Committee

The ESC will be established and will operate as described in MSA §3. Baptist Health's members will include its Executive Champion, Technology Lead, clinical and/or operational Subject Expert(s), and Project Lead. Either Party may call ad-hoc meetings as provided in MSA §3.4.

B. Baptist Health Personnel

Workgroups will operate as described in MSA §3.5. Baptist Health will identify the following resources. The level-of-effort figures are estimates based on Vega Health's prior experience with similar engagements; any significant changes or re-scoping will be discussed with the Executive Champion and the ESC.

Figure 5. Anticipated Baptist Health resources and responsibilities

Role	Responsibilities
Executive Champion	Single point of contact and decision-maker for the engagement (MSA §3.3); ESC member
Technology Lead	Technical expertise to enable safe and secure Platform installation within Baptist Health environment; provision of technical resources for Platform installation and Use Case-specific monitoring capabilities
Project Lead	Project Plan, reporting, acceptance packages, risk management, and day-to-day operations in collaboration with Vega Health Engagement Manager
Use Case Subject Expert(s)	Clinical or operational expertise for each Use Case, including on data mappings, gold-standard annotations, threshold selections, and workflow designs

Role	Responsibilities
Use Case Sponsor	Clinical, technical, or operational leader in the organization who owns the delivery, maintenance, and/or use of the in-scope model for the relevant Use Case
End-Users	Workflow input, training, alert response
Health IT Analyst	FHIR and HL7 integrations; Platform maintenance support
Infrastructure & Storage Architect	Environment provisioning and change management
Information Security Analyst	Security review and validation
Data Analyst	Data governance compliance and reporting
Networking Analyst	Networking, DNS, and VPN configuration

C. Vega Health Personnel

The Vega Health Engagement Manager will be the primary point of contact throughout the Pilot Term and will co-facilitate workgroup meetings with the Baptist Health Project Lead. Additional Vega Health personnel may be included, as needed, or as defined in the Project Plan.

Figure 6. Vega Health resources and responsibilities

Role	Responsibilities
Executive	ESC co-chair
Engagement Manager	Project Plan, Deliverables, risk management, and day-to-day operations in collaboration with Baptist Health Project Lead
Forward Deployed Engineer	Coordination of Platform installation, including site adapter, data pipelines, and troubleshooting; Use Case-specific technical support
AI Scientist	Scoping and feasibility testing for Monitoring Plans; Use-Case onboarding; implementation of monitoring capabilities

D. Meeting Frequency

Touchpoints are anticipated to include: (1) at minimum, monthly ESC meetings; (2) biweekly meetings with the Executive Champion; (3) weekly meetings with the Project Lead; (4) regular meetings with technical personnel (e.g., Data Analyst, Networking Analyst); and (5) biweekly or monthly Use Case-specific workgroup meetings. The cadence for any meetings may be adjusted by mutual agreement of the Parties.

VII. Changes to the SOW

Changes to this SOW, including Use Case Addendum, shall be made by written amendment under MSA §4.1(a), executed by both Parties' authorized representatives.

If Baptist Health's team is affected (e.g., staff are redirected to other urgent tasks), Vega Health shall continue to perform obligations that are not dependent on Baptist Health and is excused from any resulting delay in its performance. Any necessary adjustment to the schedule, milestone dates, or Fees will be proposed by Vega Health and agreed to by the Parties.

VIII. Termination and Transition

Continued use of the Platform and monitoring of each in-scope Use Case after the Pilot Term will be documented in an Order Form under MSA §4.1(b). Platform and Use Case operational readiness documentation and trainings are provided to the ESC, Use Case Sponsor, and other identified stakeholders (e.g., end-users) before conclusion of the pilot. Upon expiration or early termination, decommissioning, availability of Data Mappings, return of Confidential Information, and transition assistance are governed by MSA §§12.3–12.4, and the return or destruction of PHI by the BAA.

Following the Pilot Term, conversion of the Platform and of each Use Case from the pilot to full implementation will occur only upon the mutual written agreement of the Parties, documented in an Order Form and/or a new statement of work or SOW Addendum setting forth the applicable scope, term, and Fees. Baptist Health is not obligated to convert every Use Case to full implementation and may elect to proceed with some, all, or none of the Use Cases, and Vega Health is not obligated to provide any services for a Use Case beyond the Pilot Term absent such written agreement. Nothing in this SOW creates any minimum financial or volume commitment, consistent with MSA §2.3.

IX. Pricing Overview

A. Details on Annual Subscriptions

Fees for continued use of the Platform and monitoring for each Use Case are charged as annual subscriptions under an Order Form, as described in MSA §4.1(b) and §4.3. Vega Health intends to work with Baptist Health so that the Platform delivers value relative to its cost; if Baptist Health reasonably determines that a specific Use Case is no longer delivering such value, the Parties will discuss in good faith identifying an alternative (e.g., a new model available through Vega Health) or offboarding the Use Case, in each case by mutual written agreement and outside the scope of this SOW. The foregoing is a statement of intent only and does not constitute a warranty or performance guarantee.

B. Payment Schedule

Milestones and the relevant Deliverables are included in the payment schedule below. Each payment is earned upon the ESC's approval of the applicable Deliverable(s) and completion of each Milestone. Vega Health will invoice upon approval, and payment terms are governed by MSA §4.3.

Figure 8. Payment Schedule

Milestone	Deliverables	Projected Timing	Amount ($ USD)
1. Install Platform	• Governance Structure • Project Plan • Vega Health Platform Installation • Technical Documentation Report	Month 2	$250,000
2. Use Case 1 Monitoring Implemented	• Use Case Addendum • Monitoring Plan • Use Case 1 Onboarding and Monitoring	Month 4	$50,000

Milestone	Deliverables	Projected Timing	Amount ($ USD)
3. Use Case 2 Monitoring Implemented	• Use Case Addendum • Monitoring Plan • Use Case 2 Onboarding and Monitoring	Month 5	$50,000
4. Use Case 3 Monitoring Implemented	• Use Case Addendum • Monitoring Plan • Use Case 3 Onboarding and Monitoring	Month 6	$50,000
5. Platform and Use Case Operational Readiness	• Operational Readiness Documentation and Trainings	Month 6	$100,000
Total			**$500,000**

X. Signatures

IN WITNESS WHEREOF, each Party hereto has caused this SOW to be executed by its duly authorized representative.

BAPTIST HEALTH	VEGA HEALTH INC.
Signature:	Signature:
Printed Name:	Printed Name: **Mark Sendak**
Title:	Title: **CEO**
Date:	Date: **xx/xx/xxxx**